

Registry analysis of hepatic enzyme elevation with Hepaquel: no identifiable individual case

Abstract. We describe the frequency of transaminase elevation among 22,000 patients dispensed Hepaquel, a fictional lipid-lowering agent, in a national pharmacy claims-linked laboratory dataset. This is an aggregate analysis and does not describe any identifiable individual patient.

Methods. Retrospective cohort. Alanine aminotransferase (ALT) values within 180 days of the first dispensing were linked. Elevation was defined as ALT greater than three times the upper limit of normal. **Methods.** This was a single-centre observational report. Data were abstracted from the electronic health record by two independent reviewers and cross-checked, with discrepancies resolved by consensus. Adverse event terms were coded to MedDRA preferred terms (version 27.0). Causality was assessed with the WHO-UMC system and, independently, with the Naranjo probability scale. Written informed consent was obtained from the patient for publication of anonymised clinical details and images. Laboratory reference ranges are those of the reporting institution's accredited laboratory. Concomitant medicines, over-the-counter products and herbal preparations were recorded where documented. Baseline organ function was reviewed and prior exposure to the suspect class was sought from the dispensing history. The report follows the CARE guideline for case reports. No external funding was received and the authors declare no competing interests. **Methods.** This was a single-centre observational report. Data were abstracted from the electronic health record by two independent reviewers and cross-checked, with discrepancies resolved by consensus. Adverse event terms were coded to MedDRA preferred terms (version 27.0). Causality was assessed with the WHO-UMC system and, independently, with the Naranjo probability scale. Written informed consent was obtained from the patient for publication of anonymised clinical details and images. Laboratory reference ranges are those of the reporting

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Results. ALT elevation above 3x ULN occurred in 1.8% of exposed patients over the observation period, compared with 0.9% in an unexposed comparator matched on age, sex and baseline liver enzymes. Elevations were largely asymptomatic and resolved on or after discontinuation. No cases of acute liver failure were identified in the dataset. Individual-level narratives were not extracted and are outside the scope of this report.

Conclusion. Hepaquel is associated with a small excess risk of asymptomatic transaminase elevation; periodic monitoring is reasonable. Because this is a population-level analysis, it contains no reportable individual case safety report. References: 1. Cohort study group. *Pharmacoepidemiol Drug Saf.* 2023.